FOR EDITH · PAEDIATRICS & CHILD HEALTH JUNIOR CLERKSHIP

Professionalism & Medical Ethics

Lecture | Week 2 · Diarrhoea, fluids, HIV & anaemia | Foundations | Dr Mawanda

Paediatric ethics is a three-way relationship between child, parents and clinician, guided by the child's best interests. In Uganda parents or guardians consent for children under 18, but children should give assent, adolescents are owed confidentiality within the limits of safety, bad news is broken with the SPIKES approach, and a parent's refusal of life-saving treatment (e.g. blood) does not override the child's right to life.

Must know before the ward round

1. The four principles (Beauchamp & Childress): autonomy, beneficence, non-maleficence and justice. In paediatrics the guiding standard is the child's BEST INTERESTS (UNCRC Art. 3; Children Act welfare principle).
2. Consent in Uganda: the age of majority is 18, so a parent or legal guardian gives consent for a child. Exceptions: emergencies (treat to save life or prevent serious harm under the doctrine of necessity), emancipated minors (married, pregnant or a parent, self-supporting), and service-specific policies (e.g. HIV testing and SRH services for adolescents) **Verify** *current age thresholds in the MoH HIV testing and adolescent SRH policies*.
3. Assent: the child's agreement, sought in age-appropriate language from about 7–8 years (UNCST research guidelines use 8 years). A child's dissent should be respected unless the treatment is essential for their life or health.
4. Valid consent needs: capacity, adequate information (diagnosis, procedure, benefits, risks, alternatives including doing nothing), voluntariness, and documentation for invasive procedures.
5. Gillick competence (UK): a child under 16 can consent if they understand the nature, purpose, risks and alternatives. Fraser guidelines apply to contraception. Uganda has no Gillick case law; know the principle and say how Ugandan practice differs.
6. Adolescent confidentiality: see the adolescent alone for part of the visit (HEADSSS), keep what they tell you confidential, and explain the limits up front: risk of serious harm to self or others, abuse (including defilement), or a legal requirement.
7. Breaking bad news: SPIKES = Setting, Perception, Invitation, Knowledge, Emotions (empathy), Strategy and Summary. Use the local language or an interpreter, avoid jargon, pause, and plan follow-up.
8. Refusal of blood by Jehovah's Witness parents: respect beliefs and minimise blood use, but in a life-threatening emergency give blood in the child's best interests, involve seniors and administration, document thoroughly, and seek a court order (High Court) where time allows.
9. Transfusion thresholds (WHO/UCG; TRACT trial 2019): transfuse when Hb <4 g/dL, or Hb 4–6 g/dL with signs of decompensation (respiratory distress, shock, impaired consciousness, hyperparasitaemia); whole blood 20 mL/kg or packed cells 10 mL/kg over 3–4 hours.
10. Medical students must introduce themselves as students, obtain consent for every examination, have a chaperone for intimate examinations, never examine intimate areas of a child without supervision, and never post patient photos or details on social media.
11. Honesty after error: disclose, apologise, explain what will be done, report through the incident system, and learn. Saying sorry is not an admission of legal liability.
12. Regulator: the Uganda Medical and Dental Practitioners Council (UMDPC) registers doctors and enforces the code of professional ethics; the Uganda Patients' Charter (2009) sets out patients' rights and responsibilities, and the Data Protection and Privacy Act 2019 treats health data as special personal data.

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1. Definitions and Key Concepts

Medical ethics is the reasoning we use to decide what we *ought* to do for patients. **Professionalism** is how we behave so that patients, families, colleagues and society can trust us.

Paediatrics is different from adult medicine in one big way: the patient (the child) usually **cannot decide alone**. Decisions are shared in a **triad**: child, parents/guardians and clinicians. The parents usually know and love the child best, so they normally decide. But their authority is limited by the child's **best interests**, and children have their own **rights** (UNCRC) and **growing autonomy**: a 4-year-old, a 10-year-old and a 16-year-old need very different conversations.

Four situations dominate exams and ward life:

1. **Consent and assent**: who can agree to what, and when you can act without consent.
2. **Confidentiality with adolescents**: what you can keep private, and when you must share.
3. **Breaking bad news**: a new diagnosis of HIV, cancer, disability, or death.
4. **Refusal of treatment**: parents who refuse blood, ART, vaccines, referral, or who want to take the child home against advice.

> **CLINICAL PEARL**
> The one thing never to forget: **when the child's life or serious health is at stake, the child's best interests come before anyone's wishes, including the parents' and your own.** In an emergency, act; in a non-emergency, talk, involve seniors, and if necessary go to the court or the Probation and Social Welfare Officer.

Key terms

Term	Meaning
Autonomy	Respecting a person's right to make informed decisions about themselves
Beneficence	Acting in the patient's best interests
Non-maleficence	"First, do no harm": avoid causing harm or exposing to unnecessary risk
Justice	Fairness: treating like cases alike, fair allocation of scarce resources (oxygen, ICU beds, blood)
Best interests	What overall benefits the child most: medical, emotional, social, spiritual, considering the child's views
Informed consent	A voluntary decision by a person with capacity, after receiving adequate information
Capacity	Ability to understand, retain, weigh information and communicate a decision
Assent	A child's agreement to a procedure, sought even when the child cannot legally consent
Dissent	A child's refusal; respected when the procedure is not essential
Emancipated minor	A person under 18 treated as an adult for consent because they are married, pregnant or a parent, or living independently (recognised in Uganda's research guidelines and some service policies)
Mature minor / Gillick competence	A child judged to have enough understanding to consent to a specific treatment (UK case law, 1985)
Confidentiality	The duty not to disclose information about a patient without their permission, except where justified
Doctrine of necessity	Treatment given without consent in an emergency to save life or prevent serious deterioration, limited to what is immediately necessary
Paternalism	Overriding a person's wishes "for their own good"; acceptable in young children's best interests, less so with mature adolescents

Term	Meaning
Futility	Treatment that cannot achieve its goal; there is no duty to provide futile treatment
Harm threshold	The point at which a parent's decision is so harmful that it falls outside what a reasonable parent could choose, so the state may intervene (section 3)

2. Burden and Uganda Context

- **Ethical conflict is daily ward work**: discharge against medical advice (DAMA), detention of patients over unpaid bills in private facilities, informal payments, stock-outs forcing rationing, one oxygen concentrator for several children, and decisions about resuscitating very preterm babies.
- **Adolescents**: about a quarter of Ugandan girls aged 15–19 have begun childbearing; many adolescents with HIV do not know their status or hide it; they need **confidential, youth-friendly** services to keep them in care.
- **Religious and cultural beliefs** shape decisions: Jehovah's Witnesses and some Pentecostal groups (blood, faith healing), traditional healers and herbal medicines (often tried first), clan elders and fathers as decision-makers (a mother may feel unable to consent alone), and beliefs about disability.
- **Law and regulation**: the Constitution, the Children Act, the Medical and Dental Practitioners Act and UMDPC Code of Professional Ethics, the Patients' Charter, and acts on HIV, mental health and data protection all bear on everyday decisions (summarised in section 7).

> **UGANDA CONTEXT**
> Who can consent in practice? On Ugandan wards the **mother** usually brings the child and signs, but families may expect the **father or an elder** to decide on major procedures. Legally, either parent (or a legal guardian) may consent; a relative who is caring for the child (e.g. a grandmother) is accepted in practice. If no one with parental responsibility is available and the child needs urgent treatment, **treat under necessity** and document. For non-urgent procedures, contact the parent or involve the **PSWO**.

Support available by level of care

Resource	When	What it offers	How it changes management	Availability
Senior clinician / consultant	Any significant dilemma	Experience, authority, shared responsibility	Decision made at the right level	All hospitals
Medical social worker	Financial barriers, family conflict, safeguarding	Family assessment, links to support	Removes reasons for refusal (cost, transport)	Most hospitals
Counsellor / psychologist	Bad news, disclosure, grief	Ongoing emotional support	Better coping and adherence	Hospitals, ART clinics
Chaplain / religious leader	Faith-based refusal	Faith-sensitive dialogue	May find acceptable compromises	Mission hospitals (Mengo has chaplaincy)
Hospital ethics committee / administration	Refusal of life-saving treatment; end-of-life decisions; resource allocation	Institutional decision and support	Legal and institutional backing	Teaching and referral hospitals
Probation and Social Welfare Officer (PSWO)	Neglect, abuse, parents unavailable or unfit	Statutory child protection; care orders	Legal authority for decisions	Every district

Resource	When	What it offers	How it changes management	Availability
Courts (High Court; Family and Children Court)	Persistent refusal of essential non-emergency treatment	Court order authorising treatment	Legal authority to treat	Kampala and regional centres
Palliative care team	Life-limiting illness	Symptom control, family support	Goals-of-care plan	Hospice Africa Uganda, Mulago, Mengo, regional teams
UMDPC	Professional misconduct	Regulation and discipline	Accountability	National

3. Principles and Framework

The four principles (Beauchamp & Childress)

Autonomy, beneficence, non-maleficence and justice (definitions in section 1). In paediatrics, autonomy is shared and growing, so the principles are weighed through the child's **best interests** (UNCRC Art. 3; the Children Act welfare principle), with the child's views given weight according to age and maturity (UNCRC Art. 12).

The four-quadrant approach (Jonsen, Siegler & Winslade)

A structured way to analyse any dilemma on the ward:

Quadrant	Questions
Medical indications (beneficence, non-maleficence)	What is the diagnosis and prognosis? What are the treatment options, benefits and risks? Is it urgent?
Patient preferences (autonomy)	What does the child want (assent/dissent)? What do the parents want and why? Is the decision-maker informed and free?
Quality of life	What will the child's life be like with and without treatment? Whose judgement is it?
Contextual features (justice, law, culture)	Family, religion, culture, finances, law (Children Act), resources, conflicts of interest

The child's developing autonomy

Age (guide)	Role in decisions
Under about 7	Inform simply; distract and comfort; parents decide
About 7–12	Seek **assent**; explain in concrete terms; respect dissent for non-essential procedures
About 12–17	Increasing capacity; involve fully; see alone; confidentiality; may consent to some services (HIV testing, SRH) per national policy; refusal of life-saving treatment is still not decisive
18+	Adult: full autonomy

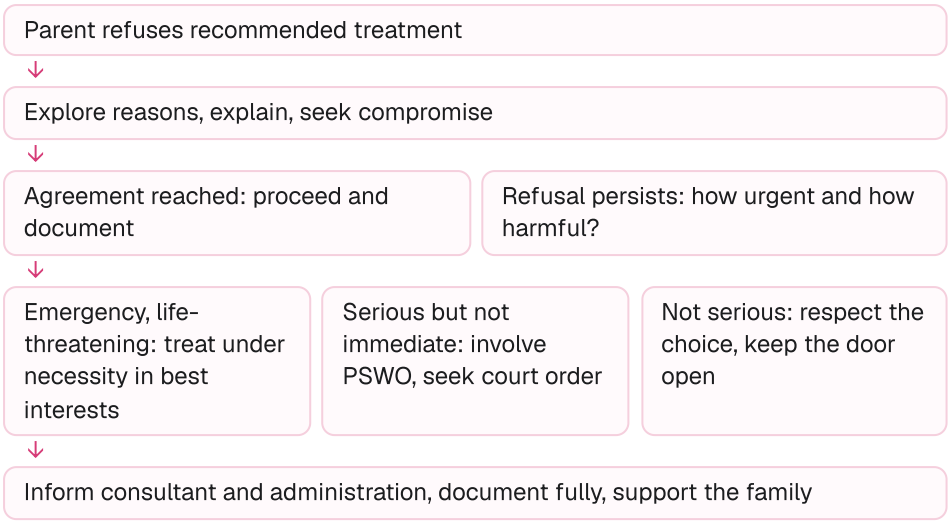
Valid consent

Four elements (**CIVD**): **capacity**, adequate **information** (diagnosis, procedure, benefits, risks, alternatives including doing nothing), **voluntariness**, and **documentation** for invasive procedures. Capacity means the person can **understand, retain, weigh** the information and **communicate** a decision (**URWC**).

Why best interests can override parents

Parents' rights exist to let them fulfil their **responsibilities** to the child. When a parent's decision would expose the child to **serious harm** (death, disability) that could be avoided, the state (through courts, the PSWO and health workers) may intervene. This is the **harm threshold**: the question is not "Is this the best decision?" but "Is this decision so harmful that it falls outside what a reasonable parent could choose?"

Urgency decides the pathway



4. Application in Practice

The "workup" of an ethical problem follows the same step-by-step logic as a clinical one.

Step 0: First 5 minutes

1. **Is it an emergency?** If life or limb is at immediate risk, give essential treatment under the **doctrine of necessity** (e.g. oxygen, glucose, antibiotics, fluids, and blood if Hb is critically low with decompensation), while calling seniors.
2. **Is the child safe?** If abuse is suspected, protect first (see *Child Rights & Protection*).
3. **Call for help**: the senior doctor on call; for refusal of life-saving treatment, inform the consultant and hospital administration early.

Step 1: Focused history (the ethical history)

Ask	Why it matters
Clinical facts: diagnosis, prognosis, options, urgency	Defines what is at stake
Who has parental responsibility? Who is the legal guardian? Who usually makes decisions?	Valid consent; who to involve
What do the parents understand about the illness and options?	Most "refusals" come from fear or misunderstanding
What are their concerns and values? Religion, past experiences, costs, traditional beliefs	Find common ground; tailor the plan
What does the child think and want? (age-appropriately, sometimes alone)	Assent, best interests
What are the consequences of delay or refusal?	Urgency determines the pathway (talk vs act)

Ask	Why it matters
Are there safeguarding concerns?	Abuse or neglect changes the duty to share and act
Legal and policy constraints (consent forms, hospital policy, national service policies)	What you may and must do
For adolescents: HEADSSS (Home, Education, Activities, Drugs, Sexuality, Suicide/depression, Safety)	Reveals what they may not say with parents present

Step 2: Focused assessment (capacity and context)

- Can the child/adolescent **understand** the information (in their language)? **Retain** it? **Weigh** risks and benefits? **Communicate** a choice?
- Is the decision **consistent** with their values, or driven by fear, pain, depression, or pressure?
- Is the parent able to decide (not intoxicated, not severely distressed, not the alleged abuser)?
- Identify pressures (cost, family elders, religious leaders).
- Look for safeguarding red flags.

Gathering the facts. There are no laboratory tests for ethics, but good decisions depend on good facts:

"Investigation"	What you are looking for	Why
Clarify diagnosis and prognosis (tests, senior review)	Certainty of benefit and harm	Weighs the urgency of overriding refusal
Capacity assessment	Understanding, retention, weighing, communication	Whose decision it is
Family and social assessment	Decision-makers, resources, pressures	Find solutions
Safeguarding screen	Abuse, neglect	Changes duties to share and act
Policy and legal check (consent forms, hospital policy)	What is required	Protects child, family and staff

Step 3: Bedside tools

Tool	Use
Four-quadrant approach	Structured analysis of any dilemma
SPIKES	Breaking bad news
HEADSSS	Adolescent psychosocial history and confidential space
Teach-back	Checking understanding for informed consent
Interpreter (not a child sibling)	Accurate communication in the family's language
Consent form with documented discussion	Invasive procedures, surgery, research

Step 4: Bring in the support systems

Use the resources in section 2 at the right time: the senior or consultant for any significant dilemma, the social worker for cost and family conflict, the chaplain for faith-based refusal, the ethics committee or administration for refusal of life-saving treatment and end-of-life decisions, the PSWO for neglect or absent parents, the court for persistent refusal of essential non-emergency treatment, and the palliative care team for life-limiting illness.

Step 5: Putting it together

Model one-liner: "A 5-year-old girl (16 kg) with severe malaria and Hb 3.2 g/dL, respiratory distress and reduced consciousness, whose Jehovah's Witness parents refuse transfusion; she will probably die without blood within hours."

Worked analysis (four quadrants):

- *Medical*: transfusion is life-saving and urgent (Hb <4 with decompensation); alternatives do not work fast enough.
- *Preferences*: parents refuse on religious grounds; the child is too young to form an independent view.
- *Quality of life*: full recovery expected with treatment.
- *Context*: the Constitution protects the right to life; the Children Act puts the child's welfare first; this is an emergency.

Decision: explain respectfully, involve the consultant and administration, document, and **transfuse** under necessity in the child's best interests; continue to respect the family's faith in every other way (blood-sparing measures, privacy, chaplaincy), and do not punish or shame them.

Model case presentation (child):

> "Kato is a 6-year-old boy from Mukono, weight 17 kg (WAZ −1.1), height 112 cm (HAZ −0.6), fully immunised for age, HIV-negative, still eats family food with good appetite, admitted with severe malaria and severe anaemia. On arrival he had Hb 3.4 g/dL, RR 54, SpO_2 91%, cold peripheries and was responding only to voice. His parents are Jehovah's Witnesses and declined transfusion after a full explanation by the consultant. Because his life was in immediate danger, the consultant authorised transfusion under the doctrine of necessity in his best interests, after informing hospital administration; we gave whole blood 20 mL/kg over 4 hours, IV artesunate 3 mg/kg and oxygen, and documented the discussions in full. He has improved: Hb 6.8 g/dL post-transfusion, alert, RR 32. The family remain distressed; we have offered chaplaincy and agreed to avoid further blood products unless life-threatening, and to minimise blood sampling."

5. Clinical Acumen

Thinking like a clinician

- **Most refusals are negotiable.** Ask *why*. Cost? Fear of the procedure? A bad experience? Religious belief? Explain, address the specific fear, offer alternatives, and give time when the clinical situation allows.
- **Separate the urgent from the important.** A child in shock with Hb 2.8 g/dL cannot wait for a court; a child with Hb 6.5 g/dL and no distress can (TRACT trial: no benefit from immediate transfusion in uncomplicated Hb 4–6).
- **Assent matters even when it isn't legally required**: it reduces fear and builds trust. Explain to the 6-year-old what the cannula will feel like.
- **Confidentiality protects adolescents' health.** If teenagers think you will tell their parents about sex, contraception or HIV, they will stop coming.
- **Never promise unconditional secrecy.** Promise confidentiality *with limits* and explain them at the start.
- **Ask for help.** Ethical decisions with major consequences are made by the senior team, sometimes with the hospital ethics committee, administration or legal advisers, never by a clerk alone.

Red flags

> **RED FLAG**
>
> - Parents refuse **life-saving treatment** (blood in severe anaemia, antibiotics in meningitis, surgery for an obstructed bowel, ART in an infant).
> - Parents want to **take a critically ill child home** against advice (to a healer or because of cost).
> - An adolescent discloses **abuse, suicidal thoughts or defilement**.
> - A parent asks you to **hide a diagnosis** (HIV, cancer) from a teenager who is asking directly.
> - Pressure to **falsify records** (age, cause of injury, birth date) or accept **informal payments**.
> - A colleague who is **impaired** (alcohol) or repeatedly unsafe.

Situations that look alike but need different responses

Situation	Key distinguishing feature	Right response
Refusal from misunderstanding	Parents don't understand the illness or the procedure	Explain, use teach-back, interpreter, time
Refusal from fear or cost	Specific fear (death on the table) or fees	Address the fear; social worker; waivers
Refusal from belief (religion, culture)	Stable values, informed	Respect where possible; best interests override only when harm is serious
Refusal as neglect	Repeated failure to give essential care despite support	Safeguarding referral (PSWO)
Adolescent refusal	Mature teenager declines; capacity assessed	Explore reasons; respect for non-essential care; life-saving care still given
Request to withhold information	Parent asks you not to tell the child	Explore reasons; plan age-appropriate disclosure together; do not lie
Confidentiality vs safety	Adolescent discloses risk	Share only what is needed with those who need to know

Common student mistakes

- Treating "the parents refused" as the end of the conversation instead of the start of one.
- Promising a teenager total secrecy, then having to break it.
- Delivering bad news in the corridor, standing, with jargon.
- Using a sibling as interpreter.
- Writing findings or vital signs you did not personally obtain.
- Examining without asking, or doing an intimate examination without a chaperone and supervision.

6. Common Problems and Their Management

A. Obtaining valid consent (step by step)

1. **Who**: the parent or legal guardian (or an emancipated minor; or the adolescent where national policy allows, e.g. HIV testing, contraception).
2. **Where and when**: a private place, not when the parent is in acute distress if it can wait; never immediately after giving sedatives.
3. **What to explain**: the diagnosis, the proposed procedure and why, expected benefits, **common and serious risks**, **alternatives** (including doing nothing), who will do it, and what happens afterwards.

4. **How**: in the family's language, without jargon, with diagrams if useful; check understanding (**teach-back**); invite questions.
5. **Voluntariness**: no pressure, threats or bills used as leverage.
6. **Assent**: explain to the child in age-appropriate words; seek agreement; acknowledge fear.
7. **Document**: signature or thumbprint, witness, date, and a note of the discussion.
8. **Emergency exception**: if delay would risk life or serious harm and no one can consent, give the necessary treatment and document why.

> NOTE
> As a **medical student**, you may obtain consent only for history and examination (and only for procedures you are competent and supervised to do). Introduce yourself as a student, ask permission, and accept a "no" gracefully. Intimate examinations of children (genital, rectal, breast) require a chaperone and supervision.

B. Confidentiality with adolescents

1. At the start, explain: "What we talk about stays between us, unless I'm worried that you or someone else could be seriously hurt. If that happens, I'll tell you first."
2. See the adolescent **alone** for part of the visit; use **HEADSSS**.
3. Encourage (but do not force) them to involve a parent or trusted adult.
4. **Break confidentiality only** when there is risk of serious harm (suicide, abuse, defilement, violence), a legal requirement (court order, notifiable disease), or with consent. Share the **minimum** necessary, with the **right people**, and tell the young person.
5. Protect records: no patient details on **WhatsApp groups or social media**; follow the **Data Protection and Privacy Act 2019**.
6. HIV status: disclosure to partners is encouraged and supported; the HIV Prevention and Control Act 2014 allows a medical practitioner to disclose to a partner at real risk in limited circumstances; take senior advice.

C. Breaking bad news: SPIKES

Step	What to do (paediatric example: a new diagnosis of HIV, cancer or a child's death)
S: Setting	A private room, sit down, phone silenced, tissues, both parents or the people the mother wants with her, an interpreter if needed, a nurse/counsellor present, enough time
P: Perception	"What have you been told so far? What do you think is happening?"
I: Invitation	"Would you like me to explain the results in detail?" Respect how much they want to know
K: Knowledge	Give a warning shot ("I'm afraid the results are not what we hoped"), then the news in plain words, in small chunks, without jargon; pause; check understanding
E: Emotions / Empathy	Allow silence and crying; name the emotion ("I can see this is a terrible shock"); respond with empathy, not more facts
S: Strategy and Summary	Summarise, outline the plan and next steps, offer hope where honest (treatment, support), arrange follow-up and contacts, document what was said

When a child dies: tell the parents together, in private, using the word "**died**" (not "we lost him"); let them see and hold the child; respect religious and cultural rites (washing, prayers, rapid burial); explain any post-mortem or legal requirements; help with practical matters (body release, transport); offer bereavement follow-up; and look after the team (debrief).

D. Refusal of treatment

General approach

1. Explore reasons (fear, cost, belief, past experience, misinformation).
2. Explain the consequences honestly and check understanding.
3. Seek compromise and alternatives that are acceptable and safe.
4. Involve seniors, the social worker, a chaplain/religious leader, family elders (with the parents' permission).
5. Decide by **urgency and harm**:
   - **Emergency, life-threatening**: treat under necessity in the child's best interests; document.
   - **Serious but not immediate**: keep negotiating; if refusal persists and the child will be seriously harmed, involve the **PSWO** and seek a **court order**.
   - **Not serious**: respect the parents' choice; keep the door open.
6. **Document** everything: discussions, who was present, the information given, the decision and the reasons.
7. **Maintain the relationship**: the family will still need you.

Specific scenarios

Scenario	Approach
Jehovah's Witness parents refuse blood	Respect beliefs; minimise blood loss and blood tests; treat the cause (malaria, iron deficiency); transfuse **only if truly necessary** (WHO/UCG thresholds); in an emergency with decompensation, transfuse under necessity; in non-emergency cases with predictable need (e.g. surgery, chemotherapy), seek a court order early; involve the hospital liaison committee of the church if the family wishes
Parents refuse ART for an infant	Explore (stigma, denial, father unaware); counsel; involve a peer mother; untreated infant HIV is fatal, so persistent refusal is a child-protection issue (PSWO)
Vaccine refusal	Listen, correct misinformation respectfully, explain risks of disease, document; do not coerce; keep offering at every visit; in an outbreak, discuss with seniors and public health
Discharge against medical advice (DAMA)	Explore reasons (cost, other children at home, healer); offer solutions; if the child is critically ill and leaving would endanger life, involve seniors and administration (and PSWO); otherwise give the best possible plan (oral drugs, referral letter, return advice) and document the DAMA form
Adolescent refuses treatment	Assess capacity and reasons; involve parents; respect refusal of non-essential care; for life-saving care, best interests prevail
Request to withhold diagnosis from a child	Explore the parents' fears; agree a disclosure plan (e.g. HIV disclosure stages); do not lie if the child asks directly
Withholding/withdrawing life-sustaining treatment	Shared decision with parents when treatment is futile or burdensome without benefit; move to **palliative care**; never "abandon" the child
Scarce resources (one oxygen concentrator, two hypoxic children)	Justice: allocate by clinical need and likelihood of benefit using transparent criteria (e.g. SpO_2, ETAT priority); use splitters; escalate the shortage; document

E. Drugs and doses you will need in these cases

Indication	Drug / product	Dose	Route	Frequency	Duration	Max / notes
Severe anaemia needing transfusion (Hb <4, or 4–6 with decompensation)	Whole blood (or packed red cells)	**20 mL/kg** whole blood (TRACT 2019: **30 mL/kg** reduced mortality in afebrile children, but not in febrile ones) or **10 mL/kg** packed cells	IV	Over **3–4 hours**	Once; re-check Hb	Furosemide 1 mg/kg IV only if fluid overload

Indication	Drug / product	Dose	Route	Frequency	Duration	Max / notes
Blood-sparing in a family refusing blood	Ferrous sulphate (elemental iron)	**3 mg/kg/day** (therapeutic 3–6 mg/kg/day)	Oral	Once daily (or divided)	3 months	Max 200 mg elemental/day; start after acute infection is treated
	Folic acid	**2.5–5 mg**	Oral	Once daily	1–3 months	
	Treat malaria: artesunate	**3 mg/kg** (<20 kg), 2.4 mg/kg (≥20 kg)	IV/IM	0, 12, 24 h then daily	Until oral ACT tolerated	Removes the cause of haemolysis
	Oxygen	To keep SpO_2 ≥94% when distressed	Nasal prongs	Continuous	While hypoxic	Supports oxygen delivery
Pain in palliative care (WHO 2012 paediatric persisting pain)	Paracetamol	**15 mg/kg**	Oral	6-hourly	As needed	Max **60 mg/kg/day**
	Oral morphine (immediate release)	1–12 months: **80–200 µg/kg**; 1–2 years: **200–400 µg/kg**; 2–12 years: **200–500 µg/kg** (start low)	Oral	**4-hourly**	Titrate to pain	Maximum **starting** dose 5 mg (2–12 years) (WHO 2012); free oral morphine solution in Uganda
Breathlessness at end of life	Oral morphine	25–50% of the analgesic dose	Oral	4-hourly as needed	–	Fan, positioning
Grief/acute distress (parents)	Not drugs: counselling and support	–	–	–	–	Avoid sedating bereaved parents routinely

DRUG DOSING

WHO withdrew its 2012 guideline on persisting pain in children in 2019 and replaced it with the 2020 guideline on chronic pain in children; the morphine starting doses above are still those used in paediatric palliative care practice Verify *current dose table in the Uganda palliative care guidelines and the latest WHO guidance*. Always weigh the child and start low.

UGANDA CONTEXT

Uganda has been an African leader in palliative care since **Hospice Africa Uganda** (1993) pioneered affordable **oral morphine solution**, made in-country and distributed free in the public sector. Nurses and clinical officers with special training can prescribe it. There is no reason for a dying Ugandan child to be in uncontrolled pain.

F. Professional conduct for clerks and doctors

1. **Introduce yourself** (name and role) and wear your ID.
2. **Respect**: address caregivers by name, sit at the bedside, explain before touching, protect privacy (curtains, covering), respect culture and religion.
3. **Honesty**: never fabricate findings, observations or vital signs; if you didn't do it, don't write it.
4. **Work within competence**; ask for supervision; escalate early.
5. **Documentation**: legible, dated, timed, signed, factual; record discussions with families.
6. **Teamwork**: respect nurses, clinical officers and support staff; clear handovers (e.g. SBAR).
7. **Errors**: disclose to seniors and family, apologise, report in the incident system, learn without blame.
8. **Money and gifts**: no informal payments; small gifts of gratitude may be acceptable but never as a condition of care; declare conflicts of interest (drug companies, private referrals).
9. **Social media**: no photos or identifiable details of patients, even "anonymised" ones from a recognisable ward.
10. **Boundaries**: no romantic or sexual relationships with patients or carers; chaperones for intimate examinations.
11. **Self-care**: fatigue and burnout cause errors; seek help for stress, substance misuse or depression; report an impaired colleague through seniors.
12. **Research**: any study on children needs ethics committee approval (UNCST-registered IRB), parental consent and child assent.

Examining a child (Hutchison's approach applied ethically). Introduce yourself and your role, obtain the parent's consent and the child's assent, wash your hands, position the child (often on the parent's lap) and expose only what is needed while keeping the child warm and covered, observe before touching, leave distressing parts (ears, throat) to the end, use a chaperone for intimate examinations, thank the family and wash your hands again.

G. When ethics and professionalism fail (complications)

- **Harm to the child**: preventable death or disability (delayed transfusion, DAMA), pain, psychological harm from deception or careless bad-news delivery.
- **Loss of trust**: families avoid facilities; adolescents disengage from HIV care after a breach of confidentiality.
- **Legal and regulatory**: UMDPC disciplinary action, civil claims, criminal liability (e.g. failure to report defilement, falsified records).
- **Moral distress and burnout** in staff who are forced to act against their values (stock-outs, rationing).
- **Inequity**: unfair allocation of scarce resources, discrimination by tribe, wealth, disability, HIV status.

7. Guidelines, Policy and Law in Uganda

Instrument	What it means for you
Constitution of Uganda (1995)	**Art. 22**: right to life; **Art. 34**: rights of children
Children Act (as amended 2016)	**Welfare principle**: the child's best interests come first; parental responsibility; role of the PSWO; care orders through the Family and Children Court
UN Convention on the Rights of the Child	**Art. 3**: best interests; **Art. 12**: the child's views given due weight
Medical and Dental Practitioners Act and the **UMDPC Code of Professional Ethics**	Registration, standards of conduct, discipline
Uganda Patients' Charter (2009)	Patients' rights (information, consent, privacy, dignity, complaint) and responsibilities

Instrument	What it means for you
HIV Prevention and Control Act (2014)	HIV testing, confidentiality, and limited disclosure to a partner at real risk
Mental Health Act (2018)	Consent and treatment for mental illness
Data Protection and Privacy Act (2019)	Health data are **special personal data**: collect, store and share only what is needed
UNCST National Guidelines for Research Involving Humans	Ethics approval, parental consent and child assent (from 8 years) for any study on children Verify *current edition*
Uganda Clinical Guidelines 2023	Transfusion thresholds; HIV testing and consent; sexual assault
Consolidated HIV and AIDS Guidelines (2022)	Adolescent consent to HIV testing, disclosure to children and partners

UGANDA CONTEXT

UK law versus Ugandan practice. Gillick competence (a child under 16 with sufficient understanding may consent; House of Lords, 1985) and the Fraser guidelines for contraception are UK case law, not Ugandan law. Uganda has no Gillick case law: the age of majority is 18 and parents consent, with exceptions for emergencies, emancipated minors and specific services (HIV testing, adolescent SRH) set out in MoH policy. In an exam, state the principle and then say how Ugandan practice differs.

UGANDA CONTEXT

Transfusion: UCG versus the TRACT trial. UCG 2023 and the WHO Pocket Book use 20 mL/kg whole blood (or 10 mL/kg packed cells) over 3–4 hours. TRACT (2019, including Ugandan sites) found 30 mL/kg reduced mortality in **afebrile** children but not in febrile ones, and that immediate transfusion gave no benefit in uncomplicated Hb 4–6 g/dL. Follow UCG and discuss the volume with your senior.

8. Key Points

High-yield points

- The child's **best interests** are the guiding standard; parents decide within that limit (the harm threshold).
- **Emergency + no consent = treat under necessity**, limited to what is immediately needed, and document.
- Courts are for **non-emergency** persistent refusal; a dying child cannot wait.
- Seek **assent** from about 7–8 years; respect dissent for non-essential care.
- See adolescents **alone**, use **HEADSSS**, and explain the limits of confidentiality **before** they talk.
- Break bad news with **SPIKES**, in private, sitting down, in the family's language.
- Most refusals come from **fear, cost or misunderstanding**: ask why first.
- **Never** fabricate findings, post patient images, or accept informal payments.
- After an error: **disclose, apologise, report, learn**.
- Oral morphine is free in Uganda: no child should die in uncontrolled pain.

Numbers & doses

Item	Value
Age of majority / consent in Uganda	**18** years
Assent sought from about	**7–8** years (UNCST research: 8 years)
Gillick (UK)	Under **16** with sufficient understanding

Item	Value
Transfusion threshold (WHO/UCG)	Hb **<4 g/dL**, or **4–6 g/dL** with decompensation
Transfusion volume	Whole blood **20 mL/kg** (30 mL/kg in afebrile children, TRACT) or packed cells **10 mL/kg**, over **3–4 h**
Furosemide (fluid overload during transfusion)	**1 mg/kg** IV
Oral morphine (2–12 years)	**200–500 µg/kg** 4-hourly (max starting dose 5 mg), titrate
Oral morphine (1–12 months / 1–2 years)	**80–200 µg/kg** / **200–400 µg/kg** 4-hourly
Paracetamol	**15 mg/kg** 6-hourly, max **60 mg/kg/day**
Ferrous sulphate (treatment)	**3–6 mg/kg/day** elemental iron for 3 months (max 200 mg/day)
Folic acid	**2.5–5 mg** daily
UNCRC best interests	**Article 3**; child's views **Article 12**
Constitution	Right to life **Art. 22**; children's rights **Art. 34**
Uganda Patients' Charter	**2009**
HIV Prevention and Control Act	**2014**
Mental Health Act	**2018**
Data Protection and Privacy Act	**2019**
Hospice Africa Uganda (oral morphine)	**1993**

Exam traps

- **"The parents refused, so nothing can be done."** Wrong in an emergency: the child's right to life comes first; treat under necessity.
- **"Get a court order before giving blood to a dying child."** Wrong: courts are for **non-emergency** persistent refusal; emergencies cannot wait.
- **"Tell the parents everything the adolescent said."** Wrong: adolescents are owed confidentiality within limits.
- **"Promise the child you'll keep the secret."** Wrong: never promise unconditional secrecy.
- **"Tell the bad news quickly in the corridor so the family isn't kept waiting."** Wrong: private setting, SPIKES, time.
- **"A sibling can interpret."** Avoid children as interpreters, especially for bad news or sensitive topics.
- **"Lie to the 12-year-old who asks if he has HIV because his mother asked you to."** Wrong: don't lie; plan disclosure with the mother promptly.
- **"An apology admits legal liability."** No: honest disclosure and apology are professional duties.
- **"Gillick competence applies in Uganda."** It is UK case law; know it, then state the Ugandan position.

Questions seniors ask

Viva questions

1. *Name the four principles of medical ethics.* Autonomy, beneficence, non-maleficence, justice.
2. *Who can consent for a 10-year-old in Uganda?* A parent or legal guardian; in an emergency treatment proceeds under necessity; the child's assent should be sought.
3. *What is Gillick competence?* A UK legal test allowing a child under 16 with sufficient understanding to consent to treatment; Fraser guidelines apply to contraception.

4. *What are the limits of confidentiality for an adolescent?* Serious risk of harm to self or others, abuse (including defilement), legal requirement; otherwise keep it confidential.
5. *What does SPIKES stand for?* Setting, Perception, Invitation, Knowledge, Emotions, Strategy/Summary.
6. *Parents refuse blood for a child with Hb 3 and respiratory distress. What do you do?* Explain, involve the consultant and administration, transfuse under necessity in the child's best interests, document, and support the family.
7. *What should you do if you make a drug error?* Assess and treat the child, inform the senior, disclose and apologise to the family, report the incident, learn.

On the ward round

A 15-year-old girl asks for contraception and doesn't want her mother told. What do you do? See her alone, assess maturity and understanding, screen for coercion and defilement (an older partner?), encourage involving a trusted adult, provide contraception per national adolescent SRH policy if she is competent and it is in her best interests, give condoms and STI advice, and keep confidentiality unless there is a safeguarding concern.

How do you obtain consent for a lumbar puncture in a convulsing 2-year-old? Explain to the parent why (meningitis), what it involves, risks (pain, headache, rarely infection or herniation when contraindicated), alternatives (treat empirically without LP), and get verbal or written consent; if the child is critically ill and LP can wait, start antibiotics first.

Parents want to take their child with suspected Burkitt lymphoma to a traditional healer. How do you respond? Respect their beliefs, explain that Burkitt lymphoma is highly curable with chemotherapy (often free at the Uganda Cancer Institute) and fatal without it, explore cost and fears, involve the social worker and possibly the healer or a religious leader, agree that they can use prayer or traditional support alongside treatment, and if they still refuse, discuss with seniors (child protection/court).

How would you break the news of a new HIV diagnosis to a 14-year-old? With the caregiver's agreement and a counsellor, in private, using SPIKES; explain in simple terms that HIV is a long-term but controllable condition, that treatment allows a normal life and that undetectable means untransmittable; address fears; plan follow-up and peer support; discuss who to tell.

Two children need the last oxygen concentrator. How do you decide? By clinical need and likely benefit (lowest SpO_2, emergency signs, response expected), using transparent criteria, splitters if possible, reassessing frequently, escalating the shortage, and documenting; not by who arrived first, who pays, or who complains most.

What makes a clerk unprofessional on a ward round? Lateness, phone use, fabricated findings, discussing patients in public spaces, photographing patients, disrespecting nurses or families, examining without consent, and not admitting uncertainty.

Mnemonics

- **The four principles: "BAN-J"**: **B**eneficence, **A**utonomy, **N**on-maleficence, **J**ustice.
- **Valid consent "CIVD"**: **C**apacity, **I**nformation, **V**oluntary, **D**ocumented.
- **Capacity "URWC"**: **U**nderstand, **R**etain, **W**eigh, **C**ommunicate.
- **SPIKES**: **S**etting, **P**erception, **I**nvitation, **K**nowledge, **E**motions, **S**trategy and summary.
- **HEADSSS**: **H**ome, **E**ducation, **A**ctivities, **D**rugs, **S**exuality, **S**uicide/depression, **S**afety.
- **Breaking confidentiality "HARM"**: **H**arm to self, **A**buse, **R**isk to others, **M**andated by law.
- **Refusal pathway "Why-Explain-Compromise-Escalate-Document" (WECED)**.

OSCE tips

- In **communication stations** (breaking bad news, consent for LP, a parent refusing treatment), **introduce yourself, check the caregiver's name and relationship**, ask what they know, deliver information in chunks,

pause for emotions, check understanding, agree a plan and **ask "What questions do you have?"** Examiners mark empathy and structure as much as facts.

- **Consent station**: cover the procedure, why, benefits, common and serious risks, alternatives including doing nothing, and check understanding with teach-back; seek the child's assent.
- **Adolescent station**: state the limits of confidentiality at the start, then use HEADSSS with the parent out of the room.
- **Refusal station**: ask why before you explain; never threaten; name the escalation path (consultant, PSWO, court) calmly if harm is serious.

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SELF-TEST

Flashcards

27 cards. Cover the right-hand column and test yourself.

#	Question	Answer
1	The four principles of medical ethics	**Autonomy**, **beneficence**, **non-maleficence**, **justice** (Beauchamp & Childress).
2	The guiding standard for decisions about children	The child's **best interests** (UNCRC Art. 3; Children Act welfare principle).
3	Who consents for a child under 18 in Uganda?	A **parent or legal guardian**. Exceptions: **emergencies** (doctrine of necessity), **emancipated minors**, and specific services where national policy lets adolescents consent (e.g. HIV testing, SRH).
4	Elements of valid consent	**Capacity**, adequate **information** (benefits, risks, alternatives incl. doing nothing), **voluntariness**, and **documentation** for invasive procedures.
5	The four components of capacity	**Understand**, **retain**, **weigh** the information, and **communicate** a decision.
6	What is assent, and from what age is it sought?	The child's agreement to a procedure even though they cannot legally consent; sought from about **7–8 years** (UNCST research guidelines: 8 years).
7	What is the doctrine of necessity?	Treatment without consent in an **emergency** to save life or prevent serious harm, limited to what is **immediately necessary**.
8	What is Gillick competence?	UK case law (1985): a child **under 16** can consent if they understand the nature, purpose, risks and alternatives of the treatment. **Fraser guidelines** apply to contraception.
9	Who is an emancipated minor?	A person under 18 treated as an adult for consent because they are **married**, **pregnant or a parent**, or **self-supporting/living independently**.
10	Limits of adolescent confidentiality	**Serious risk of harm** to self or others, **abuse** (including defilement), a **legal requirement** (court order, notifiable disease), or with consent.
11	HEADSSS	**H**ome, **E**ducation/employment, **A**ctivities, **D**rugs, **S**exuality, **S**uicide/depression, **S**afety.
12	SPIKES for breaking bad news	**S**etting, **P**erception, **I**nvitation, **K**nowledge, **E**motions/empathy, **S**trategy and summary.
13	What word should you use when telling parents their child has died?	**'Died'** (or 'dead'): clear, not euphemisms like 'we lost him' or 'he has gone'.
14	Approach to a parent refusing treatment	**Why** (explore), **Explain**, **Compromise**, **Escalate** (senior, social worker, PSWO, court), **Document**; act under necessity in an emergency.
15	Jehovah's Witness parents refuse blood for a child with Hb 3 g/dL and respiratory distress	Explain, involve the **consultant and administration**, **transfuse under necessity** in the child's best interests, document fully, respect beliefs otherwise.
16	When is a court order sought for treatment?	When parents **persistently refuse essential but non-emergency** treatment (e.g. planned surgery, chemotherapy), through the High Court/Family and Children Court, with the PSWO.
17	WHO/UCG transfusion threshold in children	Hb **<4 g/dL**, or **4–6 g/dL with decompensation** (respiratory distress, shock, impaired consciousness, hyperparasitaemia).

#	Question	Answer
18	Transfusion volume in severe anaemia	Whole blood **20 mL/kg** (TRACT: 30 mL/kg in afebrile children) or packed cells **10 mL/kg**, over **3–4 hours**.
19	The four-quadrant approach to ethical dilemmas	**Medical indications**, **patient preferences**, **quality of life**, **contextual features** (Jonsen, Siegler & Winslade).
20	Oral morphine starting dose for a 6-year-old in pain (WHO 2012)	**200–500 µg/kg** every **4 hours** (max starting dose 5 mg), titrated to pain.
21	What to do after a medication error	Assess and treat the child, **inform the senior**, **disclose and apologise** to the family, **report** the incident, learn. An apology is not an admission of liability.
22	Who regulates doctors in Uganda?	The **Uganda Medical and Dental Practitioners Council (UMDPC)**, which registers practitioners and enforces the Code of Professional Ethics.
23	Key Ugandan laws/policies on patient rights and data	**Patients' Charter (2009)**, **HIV Prevention and Control Act (2014)**, **Mental Health Act (2018)**, **Data Protection and Privacy Act (2019)**, Children Act.
24	Rules for medical students examining children	Introduce yourself as a student, get **consent**, accept refusal, **chaperone** for intimate examinations, never examine intimate areas without supervision, no patient photos or details on social media.
25	Should a sibling interpret when breaking bad news?	**No.** Use a trained interpreter or staff member; children should not interpret sensitive or bad news.
26	Allocating one oxygen concentrator between two hypoxic children	**Justice**: by clinical need and likely benefit using transparent criteria (SpO_2, emergency signs), splitters if possible, reassess, escalate the shortage, document.
27	What is the 'harm threshold' for overriding parents?	Parents' decisions are respected unless they would expose the child to **serious, avoidable harm** (death or disability), falling outside what a reasonable parent could choose.

SELF-TEST

Single best answer questions

15 questions. The answers and explanations follow at the end.

1. A 5-year-old girl (17 kg) is admitted to Mengo with severe malaria. Hb 2.9 g/dL, RR 58/min, SpO_2 90%, CRT 4 s, responds only to pain. Her parents, who are Jehovah's Witnesses, understand the risks but refuse blood. What is the MOST appropriate action?

A Inform the consultant and administration, transfuse under the doctrine of necessity in her best interests, and document the discussions

B Respect the parents' decision and give IV artesunate and oxygen only

C Apply to the High Court for an order and transfuse once it is granted

D Ask the parents to sign a discharge-against-medical-advice form

E Transfuse secretly without telling the parents

2. A 15-year-old girl attends the adolescent clinic alone. She asks for contraception, says she has a 16-year-old boyfriend at school, and does not want her mother to know. She understands the options and risks. What is the MOST appropriate response? Advanced

A Refuse because she is under 18 and needs parental consent

B Call her mother to come and consent

C Report the boyfriend to the police immediately

D See her alone, assess maturity and screen for coercion, encourage involving a trusted adult, provide contraception and condoms if she remains competent and it is in her best interests, and keep confidentiality

E Give contraception only if she agrees to stop the relationship

3. You need to break the news to a mother that her 3-year-old son has Burkitt lymphoma. Which approach is MOST appropriate?

A Tell her in the corridor straight away so she isn't kept waiting

B Give her the biopsy report to read and ask if she has questions

C In a private room with a nurse and someone she wants present, find out what she knows, give a warning shot, explain in plain language, pause for her reaction, and agree a treatment plan

D Wait until the father can come, without telling her anything in the meantime even if she asks

E Tell her the prognosis is hopeless so that she is prepared

4. A 12-year-old boy (32 kg) on ART since infancy asks you directly during a clinic visit, 'Do I have HIV? My friend said these are HIV drugs.' His mother has asked staff not to tell him yet. What is the BEST response?

A Tell him the drugs are vitamins

B Tell him everything immediately, without his mother

C Refuse to answer and change the subject

D Tell him to ask his teacher

E Acknowledge his question honestly, tell him you will talk with him and his mother together very soon, and plan full disclosure with the mother and counsellor promptly

5. An 8-year-old girl (24 kg) with suspected meningitis needs a lumbar puncture. Her mother consents but the girl is crying and says 'No, I don't want it!' She is alert, with no signs of raised intracranial pressure. What is the MOST appropriate approach?

A Abandon the LP because she has refused

B Explain to her in simple terms why it is needed, address her fear, use analgesia and comfort measures (mother present), and proceed with the mother's consent because it is important for her health

C Restrain her firmly and proceed without explanation

D Postpone antibiotics until she agrees

E Ask the mother to punish her if she does not cooperate

6. The father of a 2-year-old boy (10 kg) with severe pneumonia (SpO_2 86% on admission, now 93% on oxygen) wants to take him home to a herbalist because 'the bills are too high'. What is the BEST first step?

A Let him go and ask him to sign a DAMA form

B Lock the ward doors to prevent discharge

C Call the police

D Explore his concerns, explain that the child will probably die without oxygen, involve the social worker about fees, and offer to involve the senior and family elders

E Stop the oxygen because he has decided to leave

7. A 14-year-old boy tells you confidentially during a HEADSSS interview that he has been thinking of killing himself and has saved up tablets. He begs you not to tell his parents. What should you do?

A Tell him you must share this to keep him safe, assess risk, remove access to tablets, involve his parents and the mental health team, and tell him what you will share

B Keep it confidential as he asked

C Tell only your fellow students for advice

D Discharge him with a mental health appointment in 3 months

E Tell his school headteacher

8. Which statement about consent for children in Uganda is MOST accurate?

A Children aged 12 and over can always consent to any treatment

B Only fathers can consent for children

C A parent or legal guardian consents for a child under 18, the child's assent is sought, and emergencies may be treated under necessity

D Consent is unnecessary for any procedure in a child

E A teacher can consent for surgery during school term

9. On the ward you give a 10 kg infant 100 mg of gentamicin IV instead of the prescribed 75 mg (7.5 mg/kg). You realise the error 10 minutes later. The infant is stable. What is the MOST appropriate action?

A Say nothing because the infant is stable

B Change the chart to read 100 mg

C Blame the nurse who drew up the drug

D Tell the mother that a different drug was given

E Inform the senior at once, monitor the infant (urine output, renal function, consider levels), disclose and apologise to the mother, and complete an incident report

10. A clerk takes a photo of a child with a rare skin condition on the ward and posts it on a class WhatsApp group 'for learning', without the face showing. What is the MOST accurate statement?

A It is acceptable because the face is not visible

B It is unprofessional: images of patients need consent and secure handling, and posting breaches confidentiality and data-protection law

C It is acceptable because it is for education

D It is acceptable if the group is private

E It is acceptable if the parent did not object

11. A 6-year-old boy (18 kg) with suspected Burkitt lymphoma (jaw mass) needs a biopsy and chemotherapy. His parents want to take him to a traditional healer first, 'for 2 months'. The tumour is growing over days. After repeated counselling, the social worker's input and a fee waiver, they still refuse. He is not in immediate danger today. What is the MOST appropriate next step? Advanced

A Discharge him and hope they return

B Give chemotherapy without consent tonight under the doctrine of necessity

C Involve the PSWO and seek a court order authorising treatment, while continuing to talk with the family

D Tell the parents the child will die tomorrow to frighten them

E Accept their decision because they are the parents

12. Which of the following is a component of valid informed consent?

A Signature by a witness only

B The doctor's opinion that the treatment is beneficial

C Agreement obtained after sedation has been given

D Information limited to benefits to avoid scaring the family

E Voluntariness: the decision is made without coercion

13. A 4-year-old girl (14 kg) with relapsed, treatment-refractory leukaemia is in pain and her parents ask that she 'not be put on machines' and be kept comfortable. She has no prospect of cure. What is the MOST appropriate plan?

A Agree a palliative care plan with the parents: oral morphine 200–500 µg/kg 4-hourly titrated, paracetamol, a do-not-attempt-resuscitation decision documented, and family support

B Insist on full ICU care because withholding treatment is unethical

C Discharge home without medication because nothing more can be done

D Give only paracetamol to avoid morphine addiction

E Start a new experimental chemotherapy without discussion

14. You are the only clinician on night duty at a HC IV with one oxygen concentrator (with a flow splitter unavailable). Child A (2 years) has severe pneumonia with SpO_2 82%; child B (4 years) has asthma, SpO_2 92%, improving on salbutamol, and is currently on oxygen because he arrived first. What is the MOST ethical decision? Advanced

A Keep oxygen on child B because he arrived first

B Alternate oxygen every hour between both children

C Give oxygen to the child whose family pays the fee

D Move the oxygen to child A, continue to monitor child B closely, and escalate the shortage

E Refuse to treat either child until more oxygen arrives

15. A mother brings her 7-year-old son (21 kg) for a routine review. During history-taking you need to ask about bed-wetting, which embarrasses him in front of his older sister. Which professional behaviour is MOST appropriate?

A Ask the question loudly so that everyone in the room can hear clearly

B Ask the sister to step out (with the mother's agreement), explain why you are asking, and ask in a matter-of-fact, kind way

C Skip the question to avoid embarrassment

D Ask the sister to answer for him

E Make a joke about it to relax him

Answers & explanations

1. A · Inform the consultant and administration, transfuse under the doctrine of necessity in her best interests, and document the discussions

A is correct: Hb <4 g/dL with shock, respiratory distress and impaired consciousness is immediately life-threatening; the child's right to life and best interests justify transfusion under necessity, with senior/administrative involvement and full documentation. B is wrong: artesunate and oxygen will not correct the oxygen-carrying deficit fast enough; she is likely to die. C is wrong: a court order is for non-emergency persistent refusal; waiting hours could be fatal. D is wrong: discharging a dying child abandons her. E is wrong: deception destroys trust and is unprofessional; act openly and explain.

2. D · See her alone, assess maturity and screen for coercion, encourage involving a trusted adult, provide contraception and condoms if she remains competent and it is in her best interests, and keep confidentiality

D is correct: a mature adolescent seeking contraception should be seen confidentially, assessed for understanding and coercion, encouraged (not forced) to involve an adult, and provided with protection per adolescent SRH policy; refusing increases pregnancy and STI risk. A is wrong: national adolescent health policy allows SRH services for adolescents, and refusal causes harm. B is wrong: breaching confidentiality without a safety reason drives adolescents away from services. C is wrong: a relationship between two adolescents is legally defilement but is handled through counselling and child-protection channels with judgement, not an automatic police report, unless there is coercion, a large age gap or exploitation. E is wrong: conditional care is coercive and unethical.

3. C · In a private room with a nurse and someone she wants present, find out what she knows, give a warning shot, explain in plain language, pause for her reaction, and agree a treatment plan

C is correct: SPIKES: private setting, support present, perception, invitation, knowledge in chunks with a warning shot, empathy, and strategy (Burkitt lymphoma is highly curable with chemotherapy). A is wrong: corridors lack privacy and time. B is wrong: handing over a report without explanation is unkind and leads to misunderstanding. D is wrong: you may offer to wait for her husband, but refusing to answer her questions is not honest. E is wrong: it is inaccurate (Burkitt lymphoma has good cure rates) and destroys hope.

4. E · Acknowledge his question honestly, tell him you will talk with him and his mother together very soon, and plan full disclosure with the mother and counsellor promptly

E is correct: never lie to a child who asks directly; at 12 he is ready for full disclosure, which should be planned quickly with the mother and a counsellor so it happens supportively. A is wrong: lying destroys trust and harms adherence when he finds out. B is wrong: abrupt disclosure without preparation or his mother bypasses the planned, supported process. C is wrong: evasion increases anxiety and he may learn it badly elsewhere. D is wrong: teachers have no role in disclosure, and sharing his status breaches confidentiality.

5. B · Explain to her in simple terms why it is needed, address her fear, use analgesia and comfort measures (mother present), and proceed with the mother's consent because it is important for her health

B is correct: her dissent should be heard and her fear addressed (explanation, local anaesthetic, comfort, parent present); because the LP is important for diagnosis and treatment and the parent has consented, it can proceed with as much cooperation as possible. A is wrong: an 8-year-old's dissent does not override a necessary investigation for a serious illness. C is wrong: restraint without explanation is traumatic and disrespects her developing autonomy. D is wrong: antibiotics must never be delayed in suspected meningitis. E is wrong: threats and punishment are unethical.

6. D · Explore his concerns, explain that the child will probably die without oxygen, involve the social worker about fees, and offer to involve the senior and family elders

D is correct: most refusals are driven by cost or fear and can be resolved; explore reasons, explain the risk honestly, remove barriers (social worker, fee waiver), and escalate to seniors; if he still insists while the child is oxygen-dependent, involve administration and the PSWO. A is wrong: a DAMA form is a last resort, not a first step for a hypoxaemic child. B is wrong: detaining families is unlawful and damages trust. C is wrong: police involvement is premature and adversarial. E is wrong: stopping oxygen would harm the child and abandon him.

7. A · Tell him you must share this to keep him safe, assess risk, remove access to tablets, involve his parents and the mental health team, and tell him what you will share

A is correct: serious risk of self-harm is a recognised limit of confidentiality; explain this honestly, assess and reduce risk, involve parents and mental health services, and share only what is needed. B is wrong: keeping it secret could cost his life. C is wrong: telling uninvolved people breaches confidentiality without helping him. D is wrong: active suicidal planning needs urgent assessment, not a 3-month wait. E is wrong: the school is not the first or right channel; parents and clinicians are.

8. C · A parent or legal guardian consents for a child under 18, the child's assent is sought, and emergencies may be treated under necessity

C is correct: the age of majority is 18; parents/guardians consent, assent is sought from the child, and life-saving emergency treatment can proceed under necessity. A is wrong: there is no general rule; only certain services (e.g. HIV testing, SRH) allow adolescent consent per policy. B is wrong: either parent (or a guardian) may consent. D is wrong: consent is required except in emergencies. E is wrong: teachers have no parental responsibility for medical consent (though they may bring a child for emergency care).

9. E · Inform the senior at once, monitor the infant (urine output, renal function, consider levels), disclose and apologise to the mother, and complete an incident report

E is correct: patient safety first (monitor for nephrotoxicity/ototoxicity, adjust the next dose), then honest disclosure, apology and incident reporting so the system can learn. A is wrong: concealment is dishonest and may harm the child if monitoring is missed. B is wrong: falsifying records is serious misconduct. C is wrong: blaming others is unprofessional and prevents learning. D is wrong: lying about what happened is dishonest.

10. B · It is unprofessional: images of patients need consent and secure handling, and posting breaches confidentiality and data-protection law

B is correct: clinical images are personal (health) data under the Data Protection and Privacy Act 2019; they need explicit consent for a stated purpose and secure storage, and social-media sharing is a breach. A is wrong: the child, the bed or the ward can still identify the patient. C is wrong: an educational purpose does not replace consent and secure handling. D is wrong: 'private' groups are easily forwarded. E is wrong: absence of objection is not informed consent.

11. C · Involve the PSWO and seek a court order authorising treatment, while continuing to talk with the family

C is correct: Burkitt lymphoma is highly curable and fatal without treatment; persistent refusal of essential but non-emergency treatment is the situation for PSWO involvement and a court order, while keeping the relationship open. A is wrong: discharging without a safety plan likely leads to death. B is wrong: necessity covers immediate emergencies; chemotherapy without consent needs legal authority. D is wrong: exaggerating to frighten is dishonest. E is wrong: parental authority is limited by the harm threshold; refusing curable cancer treatment crosses it.

12. E · Voluntariness: the decision is made without coercion

E is correct: valid consent requires capacity, adequate information, voluntariness and (for invasive procedures) documentation. A is wrong: a witness signature does not replace the patient's/parent's informed agreement. B is wrong: the doctor's opinion is not consent. C is wrong: sedation impairs capacity. D is wrong: risks and alternatives must be disclosed.

13. A · Agree a palliative care plan with the parents: oral morphine 200–500 µg/kg 4-hourly titrated, paracetamol, a do-not-attempt-resuscitation decision documented, and family support

A is correct: when cure is impossible, shifting to palliative goals agreed with the family, with adequate opioid analgesia (WHO 2012 dosing) and a documented resuscitation decision, serves her best interests. B is wrong: there is no duty to give futile, burdensome treatment. C is wrong: 'nothing more can be done' is never true: symptom control and support continue. D is wrong: fear of addiction is not a reason to undertreat pain in a dying child. E is wrong: experimental treatment without consent is unethical.

14. D · Move the oxygen to child A, continue to monitor child B closely, and escalate the shortage

D is correct: justice requires allocation by need and likely benefit; child A is far more hypoxaemic, while child B is above 90% and improving; monitor B closely and escalate the shortage (and document). A is wrong: 'first come' ignores need. B is wrong: interrupting oxygen for a severely hypoxaemic child is harmful. C is wrong: allocating by payment is unjust. E is wrong: withholding care from both causes avoidable harm.

15. B · Ask the sister to step out (with the mother's agreement), explain why you are asking, and ask in a matter-of-fact, kind way

B is correct: respecting privacy and dignity (sending the sibling out with the mother's agreement, explaining, and asking calmly) is professional and gets accurate information. A is wrong: it humiliates the child. C is wrong: skipping clinically relevant questions harms care. D is wrong: the sibling is not the appropriate informant and it embarrasses him further. E is wrong: jokes about a sensitive symptom can shame the child.

SELF-TEST

Clinical cases

Work through each step before reading the model answer.

Case 1: Blood refused for a child with severe malaria

At 03:00 a 3-year-old girl (12 kg) is brought to Mengo with 3 days of fever and 1 day of fast breathing and drowsiness. Hb (HemoCue) 3.1 g/dL, RDT positive, RR 60, SpO_2 89%, CRT 4 s, responds to voice only, RBS 4.8 mmol/L. The parents are Jehovah's Witnesses and say: 'Do anything, but no blood.'

1. What clinical treatment starts immediately, whatever the decision about blood?

> **Oxygen** to SpO_2 ≥94%; **IV artesunate 3 mg/kg (36 mg)** at 0, 12, 24 h; careful fluids (no large boluses: FEAST); paracetamol 15 mg/kg; monitor glucose, consciousness and vital signs; **group and cross-match** and ask the lab to keep blood ready.

2. Is she an indication for transfusion? Justify.

> **Yes.** Hb <4 g/dL **with decompensation** (respiratory distress, shock signs, impaired consciousness). Without transfusion, mortality is very high within hours. The volume would be **whole blood 20 mL/kg (240 mL)** over 3–4 hours (TRACT: 30 mL/kg in afebrile children).

3. How do you speak to the parents?

> In private, respectfully: acknowledge their faith and love for their daughter; explain plainly that her blood is too thin to carry oxygen and she may die tonight without a transfusion; explore whether any blood-related treatment is acceptable to them; offer to call their elders or the church's hospital liaison committee; listen.

4. They still refuse. What do you do, and who do you involve?

> Call the **consultant/senior on call** and inform **hospital administration**. Because this is an emergency threatening life, the team transfuses **under the doctrine of necessity in the child's best interests** (Constitution: right to life; Children Act: welfare principle). Tell the parents openly what will be done and why.

5. What must be documented?

> Clinical findings and Hb; the indication; the information given; the parents' refusal and reasons; who was involved (consultant, administration) and when; the decision and its justification; the blood given and the child's response.

6. How do you care for the family afterwards?

> Avoid blame; continue respectful care; minimise further blood sampling; treat anaemia (iron, folic acid after recovery), prevent malaria (ITN), offer **chaplaincy/counselling**; explain that the decision was made by the team, not by them; involve the PSWO only if the family threatens to remove the child unsafely.

Take-away: Treat the cause and support the child while you talk, but when Hb is critically low with decompensation, the child's right to life overrides parental refusal: transfuse under necessity, with seniors involved, openly and fully documented.

Case 2: Confidentiality and the adolescent with HIV

A 16-year-old girl (48 kg) on TLD attends the Mengo adolescent clinic with her mother. Her viral load is 2,300 copies/mL. When seen alone, she says she has a 22-year-old boyfriend, misses doses because she hides her tablets from him, and has not told him her status. She asks you not to tell her mother anything.

1. How do you set up the confidential part of the consultation?

> Explain to both that you routinely see adolescents alone for part of the visit; then explain to her: what she says stays confidential **unless** there is a risk of serious harm to her or others or abuse, and that you would tell her first.

2. What ethical and legal issues arise from what she has told you?

> (1) **Defilement**: she is under 18 and her partner is an adult (safeguarding duty); (2) **risk of HIV transmission** to her partner (non-suppressed); (3) **non-adherence** linked to non-disclosure; (4) her **confidentiality** and autonomy; (5) possible coercion or transactional sex.

3. What will you do about the safeguarding concern?

> Explore the relationship (coercion? money? violence? where does he live?). Explain gently that, because she is under 18 and he is an adult, the law sees this as defilement and you need to involve the **social worker/PSWO** to keep her safe; tell her what you will share and with whom; involve her in the plan. Share the minimum necessary.

4. How do you address her health needs?

> **IAC** for non-suppression with practical solutions; support **partner disclosure** (assisted disclosure, couple counselling) and partner testing/PrEP; condoms; contraception (DTG has no interaction with hormonal methods); STI screen; mental-health screening; OTZ peer support; repeat VL after IAC.

5. Her mother later asks you what her daughter said. How do you respond?

> Thank her for her support, explain that adolescents are offered confidentiality because it keeps them in care, and that you share anything that puts her at serious risk. Share the **plan** relevant to adherence (with the girl's permission) and encourage the girl to talk with her mother; offer to facilitate.

> **Take-away:** Adolescent confidentiality is the default and keeps young people in care; its limits (serious harm, abuse including defilement) are explained up front, and when they apply you share the minimum with the right people and tell the adolescent first.

Case 3: Breaking bad news after a child's death

A 9-month-old boy (7.5 kg) admitted with severe dehydration and septic shock dies at 05:00 despite resuscitation on the Mengo paediatric ward. His 19-year-old mother was asked to wait outside during resuscitation; his father is on his way from Mukono. You are the intern who led the resuscitation; a nurse and a clerk (Edith) are with you.

1. How do you prepare before speaking to the mother?

> Confirm the facts (time of death, what was done); find a **private room** (or screened area), sit down; bring the nurse (and a female relative or friend the mother wants); turn off phones; decide what you will say; check whether she speaks English or Luganda and get an interpreter (a staff member, not a child) if needed.

2. How do you tell her (using SPIKES)?

> **Perception**: 'What have you understood about how your baby has been tonight?' **Warning shot**: 'I am very sorry, I have very bad news.' **Knowledge**: 'Despite everything we did, your baby **died** a few minutes ago.' Then **stop and wait**; allow crying; respond with empathy ('This is the worst news a mother can hear').

3. She asks, 'Did I do something wrong? I gave him herbs yesterday.' How do you answer?

> Honestly and kindly: explain that his illness was a severe infection with dehydration that can progress very quickly; avoid blame; acknowledge her love and that she brought him for help. If the herbs may have contributed, this is not the moment for detail; the facts can be reviewed later in a supportive way.

4. What practical steps follow?

> Offer her time with the baby (to hold him), respect cultural/religious rites (prayer, washing), wait for the father and repeat the conversation with him; explain the death certificate and body release (and whether a post-mortem or police involvement is needed: not usual here); help with transport; give a contact for follow-up; record the conversation in the notes.

5. What should happen for the team, and for learning?

> A brief **debrief** (what went well, what could be better, how people feel); check on the clerk (first death?); present the case at the **child death review** (three delays: herbs and late arrival, triage, fluids/antibiotics timing) without blame.

> **Take-away:** Tell parents of a child's death in private, clearly (use the word 'died'), with empathy and silence, without blame; then support practical and cultural needs, and look after the team and learn through a no-blame review.

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A study aid, not a protocol. Check doses against the Uganda Clinical Guidelines 2023, your unit's protocol and a senior before you treat a patient.